

MEDICAL DIAGNOSTIC REPORT

PATIENT INFORMATION

Patient Name: John Doe
Age: 45
Gender: Male
Medical Record Number: MRN123456
Date Of Visit: 2024-01-15
Doctor Name: Dr. Smith
Clinic Name: ABC Medical Center
Appointment Time: 10:30 AM
Next Appointment: In 1 week

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COMPREHENSIVE MEDICAL DIAGNOSTIC REPORT

CLINICAL ASSESSMENT

Patient presented with complaints of persistent headache and dizziness lasting for 3 days. No history of trauma or recent illness reported. Initial evaluation suggests possible tension-type headache.

PATIENT HISTORY

- Age: 45 years old
- Gender: Male
- Medical History: Hypertension, controlled with medication
- Current Medications: Lisinopril 10mg daily
- Allergies: Penicillin

PHYSICAL EXAMINATION

- Vital signs: BP 130/85 mmHg, HR 78 bpm, Temp 98.6F, RR 16/min
- General Appearance: Alert and oriented, no acute distress
- Head, Eyes, Ears, Nose, Throat: Unremarkable
- Neurological: Alert and oriented, no focal deficits noted
- HEENT: Mild tenderness over bilateral temporal regions
- Cardiovascular: Regular rate and rhythm, no murmurs
- Respiratory: Clear to auscultation bilaterally

DIAGNOSTIC FINDINGS

Based on clinical presentation and examination:

1. Tension-type headache likely secondary to stress
2. No signs of serious neurological pathology
3. Normal vital signs with mild hypertension

LABORATORY RESULTS

Complete Blood Count:

- WBC: 6.2 K/uL (Normal)
- RBC: 4.5 M/uL (Normal)
- Hemoglobin: 13.8 g/dL (Normal)

- Hematocrit: 41% (Normal)
- Platelets: 250 K/uL (Normal)

Basic Metabolic Panel:

- Glucose: 95 mg/dL (Normal)
- BUN: 14 mg/dL (Normal)
- Creatinine: 0.9 mg/dL (Normal)
- eGFR: >60 mL/min/1.73m (Normal)
- Sodium: 140 mEq/L (Normal)
- Potassium: 4.1 mEq/L (Normal)
- Chloride: 102 mEq/L (Normal)
- CO2: 24 mEq/L (Normal)

Lipid Panel:

- Total Cholesterol: 210 mg/dL (Borderline High)
- LDL: 130 mg/dL (Borderline High)
- HDL: 55 mg/dL (Normal)
- Triglycerides: 180 mg/dL (Borderline High)

TREATMENT PLAN

1. Conservative management with analgesics
 - Ibuprofen 400mg PO TID as needed for pain
 - Acetaminophen 500mg PO QID as needed
2. Lifestyle modifications
 - Stress reduction techniques
 - Regular sleep schedule
 - Adequate hydration
 - Regular exercise
3. Follow-up in 1 week if symptoms persist

MEDICATIONS

- Continue Lisinopril 10mg daily for hypertension
- Add Ibuprofen 400mg as needed for headache
- Consider prophylactic medication if headaches become frequent

RECOMMENDATIONS

- Monitor blood pressure regularly at home
- Maintain headache diary to track triggers
- Return immediately if severe symptoms develop
- Consider referral to neurology if headaches worsen
- Lifestyle modifications for hypertension control

FOLLOW-UP PLAN

- Schedule return visit in 1 week if symptoms persist
- Monitor blood pressure in 2 weeks
- Repeat labs in 3 months if indicated
- Consider specialist referral if no improvement

ASSESSMENT & PLAN SUMMARY

Likely tension-type headache with good prognosis. Conservative treatment initiated with follow-up scheduled. Patient educated on warning signs requiring immediate medical attention.

PROGNOSIS

Good with appropriate treatment and compliance to the management plan. Expected improvement within 1 week with conservative management.

ADDITIONAL NOTES

- Patient tolerated examination well
- Good understanding of treatment plan
- Will return if symptoms worsen or new symptoms develop
- Instructed on proper medication usage